

Missing the mark: Examining the status of compliance with policy standards for mental health infrastructure in Maharashtra and exploring possibilities for change

1. Introduction

India stands at a critical juncture in its mental health journey. Despite growing recognition of mental illness as a major public health concern, the country's mental healthcare system continues to rely heavily on institutional models rooted in segregation and control rather than recovery, autonomy, and community-based support. This legacy is not accidental, but reflective of a long history in which "care" for persons with psychosocial disabilities was organised through custodial asylums designed to confine and render invisible those deemed "unproductive," rather than to uphold dignity and enable healing.

International human rights standards now explicitly critique these models, highlighting how institutionalisation often leads to violations of liberty, dignity, and bodily integrity, and calling for a transition to community-based, rights-affirming systems of support.¹ International frameworks such as the UN Convention on the Rights of Persons with Disabilities (CRPD) call for an end to coercive institutionalisation and a transition toward community-based, rights-affirming systems of support. Deinstitutionalisation, peer-led care, and services embedded in everyday life are now recognised as essential to mental health justice. Yet even internationally, the shift remains uneven; community care often collapses into fragmented, underfunded services when not backed by adequate investment, leaving biomedical control intact while simply changing its location.

This global tension is deeply visible in Maharashtra. The state reflects India's broader crisis: high population-level need, persistent stigma, and a mental health system stretched far beyond capacity. Treatment gaps remain extremely high, often exceeding **70–80%**, while mental health spending continues to form only a small fraction of overall health budgets. Public infrastructure is marked by stark shortages in professionals and services, with psychiatric beds concentrated in a handful of large institutions rather than distributed through district and community care.² Judicial interventions and investigations in

¹ Committee on the Rights of Persons with Disabilities. (2023). *Concluding observations on the initial report of India (CRPD/C/IND/CO/1)*. United Nations.

<https://cdnbbsr.s3waas.gov.in/s3e58aea67b01fa747687f038dfde066f6/uploads/2023/09/20230927301902296.pdf>;

United Nations General Assembly. (2022). *Mental health: Resolution A/RES/77/300*.

<https://docs.un.org/en/A/RES/77/300>

² Sagar, R., & Singh, S. (2022). National Tele-Mental Health Program in India: A step towards mental health care for all?. *Indian journal of psychiatry*, 64(2), 117–119.; Ministry of Health and Family Welfare. (2015). *District*

Maharashtra have repeatedly highlighted systemic failures within public mental health institutions, including overcrowding, poor infrastructure, lack of safeguards, continued practices of restraint, neglect, and denial of informed consent, resulting in a system that too often defaults to confinement instead of care.³

These practices reproduce the colonial asylum's custodial logic: people are held for control and containment, often for indefinite periods, rather than supported towards recovery, autonomy, and community participation. Further, social prejudice and stigma against people labelled with mental illness and against mental health institutions themselves form a critical part of the problem when it comes to the quality of mental healthcare.⁴ Rights-based analyses by the CRPD Committee and UN Special Procedures highlight how discriminatory attitudes within families, communities, and institutions often legitimise coercion, substitute decision-making, and segregation, especially for women, older persons, queer and trans persons, and people from marginalised caste and class backgrounds.⁵ The persistence of derogatory stereotypes around “madness” and dangerousness shapes media narratives and public discourse in Maharashtra, allowing sub-standard and abusive conditions in institutions to be normalised or ignored, and deterring people from seeking early, voluntary support.⁶ Accounts from survivors, family members, and civil society organisations describe patterns of institutional violence, including physical, chemical, and seclusion-based restraint, verbal abuse, neglect, and the denial of informed consent and legal capacity.

Mental healthcare in Maharashtra is therefore not only a question of insufficient beds or workforce deficits, but also of the quality, dignity, and rights embedded in the care available. Structural underinvestment, stigma, and institutional dominance continue to prevent the emergence of trauma-informed, gender-sensitive, community-anchored alternatives. Maharashtra's public mental health system remains caught between rights-based legal

Mental Health Programme (DMHP) activities guidelines. Government of India.

<https://cmhlp.org/wp-content/uploads/2021/11/Issue-Brief-DMHP-II.pdf>

³ Nagpal, S., Grover, S., & Malhotra, S. (2019). Implications of Mental Healthcare Act 2017 for geriatric mental health care delivery: A critical appraisal. *Indian Journal of Psychiatry*, 61(Suppl 1), S107–S113.

<https://doi.org/10.4103/psychiatry.IndianJPsychiatry.11.19>

⁴ Srivastava, K., Chaudhury, S., & Prakash, J. (2018). Media and mental health. *Industrial Psychiatry Journal*, 27(2), 153–156. <https://doi.org/10.4103/ipj.ipj.82.18>

⁵ Committee on the Rights of Persons with Disabilities. (2023). *Concluding observations on the initial report of India (CRPD/C/IND/CO/1)*. United Nations.

<https://cdnbbsr.s3waas.gov.in/s3e58aea67b01fa747687f038dfde066f6/uploads/2023/09/20230927301902296.pdf>;

United Nations General Assembly. (2022). *Mental health: Resolution A/RES/77/300*.

<https://docs.un.org/en/A/RES/77/300>

⁶ Srivastava, K., Chaudhury, S., & Prakash, J. (2018). Media and mental health. *Industrial Psychiatry Journal*, 27(2), 153–156. <https://doi.org/10.4103/ipj.ipj.82.18>

commitments and an overburdened institutional reality, defined by severe resource deficits, overcrowding, and coercive practices.

This White Paper critically examines the quality of care and frameworks governing public mental health institutions in India, with particular attention to Maharashtra. It takes as its starting point the normative shift embodied in the UNCRPD, the Rights of Persons with Disabilities Act 2016, and the Mental Healthcare Act 2017, which collectively move away from a welfare and charity framework towards legal capacity, autonomy, community inclusion, and freedom from torture and cruel, inhuman, or degrading treatment. Rather than treating quality as a narrow clinical or infrastructural metric, the paper understands quality of care as inseparable from questions of power, legal capacity, intersectional discrimination, and the structural determinants that push people into institutions in the first place. Gender-sensitive and intersectional perspectives attend to how patriarchal norms, gender-based violence, unpaid care burdens, caste and class hierarchies, and queer-phobic discrimination shape both distress and access to care, demanding services that are responsive to these layered realities rather than gender-neutral in name only.⁷

Rights-based models, aligned with CRPD and MHCA 2017, centre informed consent, supported decision-making, least-restrictive alternatives, and community living, and require governments to progressively dismantle custodial institutions and invest in a continuum of community-anchored supports. By bringing these frameworks into conversation with lived realities in Maharashtra's public mental health system, including understaffing, institutional violence, and pervasive stigma, this paper investigates how the state can move from custody to care by reimagining mental health infrastructure, accountability, and community-based support in line with dignity and human rights. It proposes a model of care that centers well-being, support and argues that high-quality mental health infrastructure cannot be reduced to more beds or more psychiatrists, but must involve re-imagining how support is organised, who holds power within institutions, whose knowledge counts, and how the state honours its obligations under national and international law to respect, protect, and fulfil the rights of persons with psychosocial disabilities.⁸

2. Current Mental Health Indicators in India & Maharashtra

⁷ Ministry of Health and Family Welfare, Government of India. (2014). *National mental health policy of India*. https://www.mohfw.gov.in/sites/default/files/NMHP_0.pdf; Nagpal, S., Grover, S., & Malhotra, S. (2019). Implications of Mental Healthcare Act 2017 for geriatric mental health care delivery: A critical appraisal. *Indian Journal of Psychiatry*, 61(Suppl 1), S107–S113. <https://doi.org/10.4103/psychiatry.IndianJPsychiatry.11.19>

⁸ Kirpekar, V., Faye, A., Bhawe, S., Gawande, S., & Tadke, R. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(7), 603–613.; Ministry of Health and Family Welfare. (2025, February 6). *Advancing mental healthcare in India*. Government of India. <https://mohfw.gov.in/?q=en/pressrelease-206>

2.1 International Legal and Policy Instruments for Mental Health Rights

The following section explores the global perspectives on mental health rights by critically examining key international legal and policy frameworks.

In recent years, the recognition of mental health as a fundamental human right has gained momentum in international policy discourse. The relationship between human rights and mental health is complex and bi-directional. People with mental health conditions have historically, and continue to, face human rights violations in forms of involuntary admission, forced treatment, and prolonged institutionalisation, as well as exclusion from everyday life and restrictions on the exercise of civil and political rights. These violations occur from a deep-rooted stigmatisation associated with mental health conditions, which impacts access to appropriate, rights-affirming care.

Understanding mental health as a fundamental human right means recognising that every individual has the inherent right to the highest attainable standard of mental health, and nobody can be denied these rights on the grounds of having mental health conditions.⁹ It thus becomes imperative to examine international human rights instruments in the context of understanding the state of public mental health institutions in the state of Maharashtra. These instruments allow for a framework for the scrutiny of national mental health-related laws and policies, as well as their implementation. Moreover, products of global advocacy, they provide normative standards for liberal internationalist states. We will look into the key international declarations, treaties, principles, and guidelines that make up the backbone of the global mental health rights standards.

a. International Bill of Human Rights

The first international legal text to acknowledge health, including mental health, as a fundamental human right was the **Universal Declaration of Human Rights**.¹⁰ Adopted in 1948 by the United Nations General Assembly, it is also the first comprehensive international statement of human rights principles. India is one of the original signatory of the Declaration. Although the UDHR does not explicitly mention mental health, its provisions implicitly imply

⁹ World Health Organisation. (2023, October 6). Mental health: Promoting and protecting human rights. World Health Organization (WHO). Retrieved December 21, 2025, from

<https://www.who.int/news-room/questions-and-answers/item/mental-health-promoting-and-protecting-human-rights>

¹⁰ [Universal Declaration of Human Rights | United Nations](#)

it. According to Article 25 of the UDHR, everyone has the right to a quality of living that is sufficient for their health and well-being, including access to essential social services and medical treatment.¹¹ Article 5 further affirms that no one shall be subjected to torture or to cruel, inhuman or degrading treatment or punishment.¹² Both articles 5 and 25 are highly relevant to the mental health care provided in mental health institutions (MHI). Article 5 prohibits some of the common practices found in Mental Health Institutions (MHIs) such as conditions of confinement, use of force by explicating that treatments can also be inhuman and degrading. Article 25 extends the right to access humane and dignified care to everyone.

The UDHR is not legally binding, yet it has served as the foundational framework informing binding international treaties and national mental health legislation. In the context of MHIs, UDHR provides an entry point to rights-based practice that rejects coercive treatment, degrading living conditions, and institutional abuse. The Declaration lays down the groundwork for more explicit and enforceable standards specific to mental health, which the United Nations later developed in subsequent instruments.

b. The International Covenant on Economic, Social, and Cultural Rights

The International Covenant on Economic, Social, and Cultural Rights was adopted by the United Nations General Assembly in 1966 and entered into force in 1976.¹³ India ratified the ICESCR in 1976, committing to the right to work, health, education, and an adequate standard of living for its citizens. The treaty builds on the UDHR by making the right to health, including mental health, into a legally binding obligation. Article 12 of the ICESCR calls for the right of all individuals to the highest attainable standard of physical and mental health—explicitly referring to mental health this time. Regarding mental health and mental health institutions' rights, ICESCR obligates states to take appropriate measures to ensure access to mental health care and appropriate facilities for the treatment of mental illness. The countries that have ratified, including India, are thus legally required to implement policies that improve mental health services, integrate mental health into primary healthcare, and eliminate discrimination¹⁴ on the grounds of mental health conditions using maximum available resources.

¹¹ [Universal Declaration of Human Rights | United Nations](#)

¹² [Universal Declaration of Human Rights | United Nations](#)

¹³ [International Covenant on Economic, Social and Cultural Rights | OHCHR](#)

¹⁴ [Global Perspectives on Mental Health Rights: Laws, Policies, And Realities](#)

c. The International Covenant on Civil and Political Rights

The International Covenant on Civil and Political Rights was adopted by the United Nations General Assembly in 1966 and has been effective since 1976.¹⁵ Ratified by India in 1979, it obligates signatory states to protect fundamental rights, including freedom of speech, religion, fair trials, and freedom from torture and arbitrary detention. Article 7 states that “*no one shall be subjected to torture or to cruel, inhuman or degrading treatment or punishment. In particular, no one shall be subjected without his free consent to medical or scientific experimentation,*” making it particularly relevant to the treatment and care provided in MHIs. This provision foregrounds consent and bodily autonomy, directly in opposition to treatments that are coercive and non-consensual within mental health institutions. Article 9 and Article 17 furthermore recognise the right to liberty and security of persons and protection from arbitrary or unlawful interference with privacy, respectively. The treaty calls for realising the full capacity and agency of mental health patients receiving treatments in MHIs, along with their right to privacy and confidentiality. While the ICESCR recognises the right to access healthcare, the ICCPR goes beyond access to scrutinising the quality of the healthcare service provided. Any treatment provided must not infringe upon the human rights, dignity, and autonomy of those receiving it.

d. Convention on the Rights of Persons with Disabilities (CRPD) (2006)

The UN General Assembly adopted the **Convention on the Rights of Persons with Disabilities (CRPD)**¹⁶ in 2006—a historic UN treaty that promotes, protects, and ensures the full and equal enjoyment of all human rights and fundamental freedoms for all people with disabilities. The CRPD marks a paradigm shift from viewing people with disabilities as passive recipients of charity to “subjects” with rights who are also capable of claiming those rights. The CRPD begins Article 1 with “*Persons with disabilities include those who have long-term physical, mental, intellectual or sensory impairments...*”, explicitly recognising mental health conditions as disabilities.

This treaty, for the first time in international legal frameworks, calls for a shift away from institutionalisation toward a community-based, rights-oriented care. Informed by the social determinants of health model and disability studies, the CRPD rejects the biomedical model of disability, which constructs disability as an impairment arising in the individual that needs

¹⁵ [International Covenant on Civil and Political Rights | OHCHR](#)

¹⁶ [Convention on the Rights of Persons with Disabilities | OHCHR](#)

to be “cured.” Instead, it shifts the responsibility to governments, institutions, and organisations to remove barriers that prevent the participation of persons with disabilities on an equal basis with others.

Most importantly, Article 12 (Equal recognition before the law) recognises the inherent capacity of all persons with disabilities as equal persons before the law with the same capacity to exercise legal capacity as others. This is especially significant to people with mental health conditions because it takes a radical departure from the historical construct of people with mental health conditions as people with an unsound mind, or people who are irrational, with a lack of decision-making capacities. The treaty furthers this agency in Article 17 (Protecting the integrity of the person) by calling for the protection of the integrity of a person with disability from unwanted, non-consensual interference.

The **UN Principles of Persons with Mental Illness and the Improvement of Mental Health Care (1991)**, which served as the primary international framework governing mental health care for over a decade prior to the CRPD, operated within the ambit of custodial logic by only articulating reforms within MHIs. The CRPD, in contrast, persistently seeks to move away from institutionalisation, recognising how oppressive practices are embedded in the very architecture of such institutes, toward a community-based approach to mental health care. This is significantly apparent in Article 19 (Living independently and being included in the community), which recognises the right of people with disabilities to live independently and in communities of their own choice, and in Article 20 (Personal mobility), which calls for ensuring that they have the complete freedom and capacity to mobilise freely.

2.2 Legal & Policy Landscape

a. Current Laws & Policies in India

India’s contemporary mental health and disability framework rests primarily on three national instruments: the Mental Healthcare Act 2017 (MHCA 2017), the Rights of Persons with Disabilities Act, 2016 (RPwD Act), and the National Mental Health Policy 2014 (NMHP 2014). These are explicitly framed against India’s obligations under the Convention on the Rights of Persons with Disabilities (UNCRPD), which requires states to secure legal capacity, non-discrimination, and community inclusion for persons with disabilities, including those with psychosocial disabilities.

The NMHP 2014 frames mental health as a fundamental human right and adopts core principles of equity, social justice, respect for autonomy and dignity, and a life-course, gender-sensitive and community-oriented approach. It calls for decentralised, community-based services integrated into general health care, progressive reduction of the treatment gap through the National Mental Health Programme and the District Mental Health Programme (DMHP), and the use of institutional care only where necessary, for the shortest possible duration, and with a strong emphasis on rehabilitation and reintegration. The NMHP 2014 acknowledges the high burden of mental illness, pervasive stigma and discrimination, and the large, longstanding treatment gap, and explicitly adopts a social-determinants and human-rights lens. It prioritises integration of mental health into primary care, strengthening of the DMHP, inter-sectoral coordination (including social welfare, education, labour and justice systems), community participation, and the development of human resources and information systems. A decade-on review concludes that, although the policy has catalysed expansion of the DMHP and newer initiatives such as Tele-MANAS, implementation remains uneven: many districts continue to face shortages of trained staff and dedicated budgets, community-based services are inconsistent, and rights-based language has not yet substantially altered everyday practices within institutions.¹⁷

The RPwD Act 2016 operates as a cross-cutting equality statute. It prohibits discrimination on the basis of disability, affirms legal capacity on an equal basis with others, and recognises rights to community living, access to justice, and protection from abuse, violence and exploitation. The Act obliges the state to ensure reasonable accommodation, accessibility, inclusive education and social protection; these obligations extend to public mental health institutions and to community-based services, and provide an important legal basis for advocacy against abuse, segregation and neglect.¹⁸

The MHCA 2017 governs the organisation and regulation of mental health services and establishments, the RPwD Act provides the overarching equality and non-discrimination regime, and NMHP 2014 articulates the policy vision for promotion, prevention, treatment, rehabilitation, and social inclusion. Together with the UN Principles for the Protection of Persons with Mental Illness (MI Principles) and the 2023 UN General Assembly Resolution 77/300 on mental health and human rights, these texts define the normative environment within which states such as Maharashtra are expected to design and deliver mental health

¹⁷ Gupta, S., & Sagar, R. (2022). National Mental Health Policy, India (2014): Where have we reached? *Indian Journal of Psychological Medicine*, 14(5), 456–463. <https://doi.org/10.1177/02537176211048335>

¹⁸ Parliament of India. (2016). *The Rights of Persons with Disabilities Act, 2016* (No. 49 of 2016). https://www.indiacode.nic.in/bitstream/123456789/15939/1/the_rights_of_persons_with_disabilities_act_2016.pdf

care. The MHCA 2017 partly converts the policy commitments under the NMHP 2014 into justiciable rights. It guarantees the right to affordable, good-quality mental health care, the right to live in and be part of the community, and protection from inhuman or degrading treatment in mental health establishments. The Act introduces advance directives and nominated representatives, positions informed consent as the default basis for treatment, and requires that care be provided in the least restrictive alternative consistent with the person's needs, signalling a move away from purely custodial and carceral approaches.¹⁹ At the same time, MHCA 2017 retains the possibility of involuntary admission and treatment under specified conditions, subject to review by Mental Health Review Boards, thereby preserving a regulated space for coercion within an otherwise rights-based framework.

The MHCA 2017 establishes Central and State Mental Health Authorities (SMHAs) as statutory bodies responsible for planning, standard-setting and regulation. At state level, SMHAs must register all mental health establishments, develop and notify quality and service norms, maintain a public register, and monitor compliance with rights and safeguards set out in the Act. They are also mandated to advise the state government, coordinate with other departments, promote public awareness, and support the establishment and functioning of Mental Health Review Boards, which adjudicate complaints and review involuntary admissions.²⁰ In practice, these responsibilities have not been fully realised. In ***Dr Harish Shetty v. State of Maharashtra*** (2024), the Bombay High Court criticised the state for failing to constitute a functional SMHA, notify the necessary rules and complete registration of establishments several years after MHCA 2017 came into force.²¹ This non-implementation undermines oversight of both public and private institutions, delays effective grievance redressal, and weakens the enforceability of the rights that the Act formally guarantees.

b. Alignment with Global Standards for Mental Healthcare

The UNCRPD, the MI Principles and Resolution 77/300 articulate a global standard that rejects disability-based deprivation of liberty, calls for the dismantling of coercive and substitute-decision-making regimes, and requires states to invest in community-based supports that enable persons with psychosocial disabilities to live independently and be included in the community. On paper, India's recent reforms represent a clear movement in

¹⁹ Parliament of India. (2017). *The Mental Healthcare Act, 2017* (No. 10 of 2017). <https://www.indiacode.nic.in/bitstream/123456789/2249/1/A2017-10.pdf>

²⁰ Parliament of India. (2017). *The Mental Healthcare Act, 2017* (No. 10 of 2017). <https://www.indiacode.nic.in/bitstream/123456789/2249/1/A2017-10.pdf>

²¹ *Dr. Harish Shetty v. State of Maharashtra & Ors.*, Writ Petition No. 10742 of 2022 (Bombay High Court, decided August 2024).

this direction: MHCA 2017 and the RPwD Act adopt explicit rights-based language, recognise community living and non-discrimination, and establish oversight mechanisms such as SMHAs and Mental Health Review Boards, while NMHP 2014 centres equity, decentralisation and community-integrated care. Compared with the Mental Health Act 1987, which focused on involuntary admission and offered limited rights safeguards, this marks a significant normative advance.

Nonetheless, important tensions remain between domestic law and the UNCRPD's more expansive human-rights vision, as interpreted by the UNCRPD Committee. MHCA 2017 continues to authorise involuntary admission and treatment, and retains elements of substitute decision-making through the role of nominated representatives, whereas the Committee has consistently held that deprivation of liberty on the basis of disability and substitute-decision-making regimes are incompatible with articles 12 and 14 of the Convention. Continued reliance on long-stay custodial institutions, in the absence of adequately resourced community services, conflicts with the obligation to ensure independent living and community inclusion and with the MI Principles' emphasis on least-restrictive environments.

Implementation evidence reinforces these concerns. India's report to the UNCRPD Committee and independent assessments point to persistent under-investment in mental health, human-resource shortages, patchy District Mental Health Programme (DMHP) coverage, and ongoing reports of coercive practices, abuse and poor living conditions in institutions. Judicial interventions such as the Bombay High Court's criticism of Maharashtra for non-implementation of MHCA 2017 in the case of ***Dr Harish Shetty v. State of Maharashtra***, as well as rapid assessments of programmes like Tele-MANAS, demonstrate that formal guarantees alone do not secure compliance; sustained political will, adequate financing and participatory monitoring led by persons with psychosocial disabilities and community organisations are essential. For advocacy, this gap between law and lived reality provides a critical basis to argue for amendments that prioritise supported rather than substitute decision-making, remove disability-based detention, and redirect resources from custodial institutions towards community-anchored, trauma-informed, gender-sensitive, rights-affirming supports.

3. Literature Review

3.1 Overview

This literature review situates India's public mental healthcare system within the broader shift from custodial, institution-based psychiatric care toward an integrated, community-oriented and rights-based model envisioned in national policy frameworks. Across the last four decades, India has formally committed to embedding mental health within its general public health architecture through programmes such as the National Mental Health Programme (NMHP) and the District Mental Health Programme (DMHP). At the same time, empirical scholarship and institutional audits consistently demonstrate that the lived realities of care remain shaped by uneven implementation, biomedical dominance, resource scarcity and persistent institutionalisation.

The review is organised into three interlinked sections, each building toward the central concern of this study, namely how rights, autonomy and dignity are experienced and constrained within India's public mental health institutions. The first section maps the dominant models of mental healthcare delivery in India, tracing the evolution of the NMHP and DMHP as the principal vehicles for decentralised, community-based care. While policy discourse emphasises integration into primary healthcare and the expansion of district-level services, the literature highlights enduring reliance on hospital-centred systems, limited psychosocial and rehabilitative inputs, and growing but uneven innovations such as tele-mental health under Tele-MANAS. Taken together, this body of work suggests that India's mental health system is currently transitional, moving rhetorically toward community care while remaining structurally constrained by institutional legacies and workforce shortages.

The second section examines research and reporting on the quality of care within public mental health institutions. Here, the literature converges around four major dimensions: the material deprivation of institutional environments, the medicalised nature of treatment regimes, weak clinical governance, and pervasive experiences of coercion, constrained consent and loss of autonomy. Studies further demonstrate that institutional care is not experienced uniformly. Women and socially marginalised groups face disproportionately prolonged stays, abandonment, and gendered violations of dignity, privacy and agency. These findings are especially significant for this dissertation because they show that institutional harm is not incidental but structurally reproduced through everyday conditions of care.

The third section identifies the major faultlines that prevent the translation of progressive legislation and policy, particularly the Mental Healthcare Act 2017 and the National Mental Health Policy 2014, into meaningful structural reform. The literature consistently points to

systemic gaps in transparency and data systems, administrative delays in implementing regulatory safeguards, chronic under-financing of mental health, and weak monitoring and accountability mechanisms. These implementation vacuums allow older custodial and medication-centric practices to persist despite formal commitments to autonomy, participation and community living.

Overall, the scholarship reviewed leads to a central conclusion: India's public mental healthcare landscape is marked by a deep disconnect between rights-based policy aspirations and institutional and programmatic realities. While decentralised frameworks such as the DMHP and newer platforms like Tele-MANAS represent important steps toward widening access, the absence of adequately funded community rehabilitation, peer-led supports, gender-sensitive infrastructure and robust governance mechanisms means that many service users continue to encounter care as episodic, coercive and institutionally bounded rather than continuous, holistic and choice-driven or rooted in rights-based principles.

3.2 Public & Mental Healthcare Models Adopted in India

India's public mental health system formally rests on a three-tier public health architecture (primary, secondary and tertiary care) within which mental health is meant to be integrated rather than delivered through a separate vertical programme. The National Mental Health Programme (NMHP), launched in 1982 and revised subsequently, articulates this integrationist vision by positioning mental health services within primary health centres (PHCs), community health centres (CHCs) and district hospitals, with specialised psychiatric hospitals and academic institutions functioning as referral, training and tertiary-care hubs.²² Policy documents and World Health Organization country profiles emphasise that such integration is critical to closing India's large treatment gap and to shifting away from an asylum-based model toward community-oriented, rights-based care, particularly in states with high burdens and limited specialist resources.²³

Within this system, the District Mental Health Programme (DMHP) is widely regarded as the principal vehicle for delivering community-based mental health services through the public

²² Ministry of Health & Family Welfare, Government of India. (n.d.). *National Mental Health Programme (NMHP)*. National Health Mission.

<https://nhm.gov.in/index1.php?lang=1&level=2&sublinkid=1043&lid=359>; Srivastava, K., Nehra, R., & Kumbhar, V. (2018). National Mental Health Programme—Optimism and caution. *Indian Journal of Psychiatry*, 60(Suppl 4), S505–S511. https://doi.org/10.4103/psychiatry.IndianJPsychiatry.36_18

²³ World Health Organization. (2022). *World mental health report: Transforming mental health for all*. <https://www.who.int/publications/i/item/9789240049338>

sector. The Bellary pilot tested the onground implications of the DMPH (1985-1990) and responded to urgent mental health needs by training primary health centre (PHC) staff to identify and treat common disorders like depression, anxiety, and epilepsy using simplified protocols, reducing reliance on distant psychiatric hospitals amid India's severe psychiatrist shortage (0.75 per 100,000 population). Its popularity stemmed from cost-effectiveness, community accessibility, and demonstrated 70-80% case detection rates through decentralized delivery, influencing NMHP's national DMHP rollout.²⁴ However, it prioritized pharmacotherapy over psychosocial interventions, overlooked peer support and rehabilitation, and struggled with sustained staffing/training, prefiguring DMHP's persistent under-resourcing and biomedical bias.²⁵

The model was scaled nationally from the mid-1990s. The DMHP is designed around multidisciplinary district mental health teams that provide outpatient and short-stay inpatient services at district hospitals, run satellite clinics at CHCs and PHCs, train medical officers, nurses and health workers, and conduct school, workplace and community-level mental health promotion and suicide-prevention activities.²⁶ Standard DMHP activity guidelines specify a package that includes diagnosis and pharmacological treatment of priority mental and substance-use disorders, basic counselling and psychosocial interventions, home visits and follow-up, supply of essential psychotropic medicines and community awareness campaigns aimed at stigma reduction. Narrative and scoping reviews show that geographical coverage has expanded substantially and that the DMHP now operates in most Indian districts, but they also document marked heterogeneity in implementation, with some districts offering only periodic psychiatric camps, minimal training and little or no psychosocial or rehabilitative input beyond medication.²⁷

²⁴ Parthasarathy, R., Channaveerachari, N. K., Manjunatha, N., Sadh, K., Kalaivanan, R. C., Gowda, G. S., Basvaraju, V., Harihara, S. N., Rao, G. N., Math, S. B., & Thirthalli, J. (2021). Mental health care in Karnataka: Moving beyond the Bellary model of District Mental Health Program. *Indian journal of psychiatry*, 63(3), 212–214. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_345_19; Waraich, B. K., Raj, L., Chavan, B. S., Badhan, R., & Panda, S. (2003). Decentralisation of Mental Health Services under DMHP. *Indian journal of psychiatry*, 45(3), 161–165.

²⁵ Parthasarathy, R., Channaveerachari, N. K., Manjunatha, N., Sadh, K., Kalaivanan, R. C., Gowda, G. S., Basvaraju, V., Harihara, S. N., Rao, G. N., Math, S. B., & Thirthalli, J. (2021). Mental health care in Karnataka: Moving beyond the Bellary model of District Mental Health Program. *Indian journal of psychiatry*, 63(3), 212–214. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_345_19

²⁶ Kirpekar, V. L., Nagra, V., & Jadhav, S. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(Suppl 2), S346–S353. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_105_24

²⁷ Kirpekar, V. L., Nagra, V., & Jadhav, S. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(Suppl 2), S346–S353. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_105_24; Gangadhar, B. N., Naveen Kumar, C., Sadh, K., Manjunatha, N., Math, S. B., Kalaivanan, R. C., ... & Chaturvedi, S. K. (2023). Mental health programme in India: Has the tide really turned? *Indian Journal of Medical Research*, 157(5), 387–394. https://doi.org/10.4103/ijmr.IJMR_2217_20; Hans, G., & Sharan, P. (2021). Community-based mental

Despite the prominence of NMHP and DMHP, hospital-centred models remain structurally dominant in India's mental health landscape. Large state-run psychiatric hospitals, many of which originated as colonial asylums, continue to house sizable long-stay populations and function as the default option for people with severe mental health conditions who are homeless or lack family support and access to community services.²⁸ General hospital psychiatric units (GHPUs) have expanded across teaching and district hospitals and now provide a significant share of acute inpatient and outpatient psychiatric care, often serving as referral points for DMHP-linked primary-care facilities. Empirical studies indicate that both dedicated mental hospitals and GHPUs frequently operate within a predominantly biomedical paradigm, characterised by brief consultations, high reliance on psychotropic medication, limited structured psychotherapy and weak links to community-based rehabilitation, social entitlements or peer-support initiatives.²⁹ Reviews of the NMHP therefore describe the current Indian system as a mixed model in which progressive policy commitments to community-based care coexist with entrenched institutional practices and inadequate attention to social determinants of mental health.³⁰

Rapidly expanding literature documents a range of community-based mental health models that either complement or partially substitute for the public sector's hospital- and DMHP-centred approach.³¹ Systematic and narrative reviews identify diverse interventions,

health services in India: Current status and roadmap for the future. *Consortium Psychiatricum*, 2(3), 63–71. <https://doi.org/10.17816/CP92>

²⁸ Murthy, P. (2020). Mental hospitals in India in the 21st century: Status and role. *Indian Journal of Psychiatry*, 62(Suppl 2), S273–S276. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_131_20; Daund, M., Mistry, S., & Karia, R. (2018). Mental hospitals in India: Reforms for the future. *Indian Journal of Psychiatry*, 60(Suppl 4), S633–S635. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_104_18; Singh, O. P. (2023). A comprehensive analysis of mental health problems in India and proposed solutions. *Cureus*, 15(7), e42340. <https://doi.org/10.7759/cureus.42340>

²⁹ Chadda, R. K., & Singh, G. P. (2018). General psychiatry in India: History, scope, future. *Indian Journal of Psychiatry*, 60(Suppl 4), S518–S523. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_38_18; Pathare, S., Funk, M., Schriver, G., Jermyn, K., & Battersby, L. (2018). Peer support for mental illness in India: An underutilised resource. *Epidemiology and Psychiatric Sciences*, 27(6), 525–529. <https://doi.org/10.1017/S2045796018000169>; Venkataraman, S., & Sahoo, S. (2024). Psychiatric rehabilitation in routine Indian mental health practice. *BJPsych International*. <https://doi.org/10.1192/bji.2024.5>

³⁰ Kirpekar, V., Faye, A., Bhawe, S., Gawande, S., & Tadke, R. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(7), 603–613.; Grover, S., Dua, D., Chakrabarti, S., Avasthi, A., & Kumar, S. (2018). National Mental Health Programme–Optimism and caution: A narrative review. *Indian Journal of Psychiatry*, 60(Suppl 4), S486–S494. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_191_18; Hans, G., & Murthy, R. S. (2021). Community-based mental health services in India: Current status and future prospects. *Consortium Psychiatricum*, 2(3), 28–37. <https://doi.org/10.17816/CP142>

³¹ Hans, G., & Sharan, P. (2021). Community-based mental health services in India: Current status and roadmap for the future. *Consortium Psychiatricum*, 2(3), 63–71. <https://doi.org/10.17816/CP92>; Verma, P., Sahoo, K. C., Mahapatra, P., Kaur, H., & Pati, S. (2023). A systematic review of community-based studies on mental health issues among tribal populations in India. *Indian Journal of Medical Research*, 156(2), 291–298. https://doi.org/10.4103/ijmr.ijmr_3206_21

including village-level case-finding and referral schemes, task-sharing models in which lay health workers or community volunteers deliver basic psychosocial support under professional supervision, integration of mental health into chronic-disease and maternal-and-child-health platforms, community-based rehabilitation programmes linking clinical care with livelihood support and peer-support or self-help groups for persons with psychosocial disabilities and their families.³² Evaluations from eastern Uttar Pradesh, Gujarat and other regions show that integrated primary-care and community models can reduce symptom severity, improve functioning and enhance adherence when they provide sustained contact, home-based follow-up and practical social-support linkages rather than one-off treatment episodes.³³ Most such initiatives, however, operate at limited scale, rely on non-governmental funding or research grants and face challenges in being institutionalised within state health systems, which restricts their capacity to reshape the dominant model of care nationally.

Recent national initiatives introduce new modalities intended to widen access and partially address workforce constraints, most prominently tele-mental health services under the Tele-MANAS programme.³⁴ Tele-MANAS establishes a network of toll-free helplines staffed by trained counsellors and linked to state-level hubs where psychologists and psychiatrists can provide higher-level assessment, crisis intervention and referral, with explicit links to DMHP and other public facilities envisioned in programme design.³⁵ Government statements and early utilisation data suggest that Tele-MANAS has expanded first-contact access, especially for young people and residents of remote areas, and that it aligns with broader Ayushman

³² Hans, G., & Sharan, P. (2021). Community-based mental health services in India: Current status and roadmap for the future. *Consortium Psychiatricum*, 2(3), 63–71. <https://doi.org/10.17816/CP92>; Verma, P., Sahoo, K. C., Mahapatra, P., Kaur, H., & Pati, S. (2023). A systematic review of community-based studies on mental health issues among tribal populations in India. *Indian Journal of Medical Research*, 156(2), 291–298. https://doi.org/10.4103/ijmr.ijmr_3206_21; Kirpekar, V., Faye, A., Bhawe, S., Gawande, S., & Tadke, R. (2024).

District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(7), 603–613

³³ Shah, S. H., Desai, M., & Shah, M. K. (2021). Impact of a community-based mental health awareness programme on the knowledge, attitudes and treatment-seeking behaviour of community members in rural Gujarat, India. *Indian Journal of Psychiatry*, 63(1), 64–70. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_440_20; Jain, S., & Jadhav, S. (2008). A cultural critique of community psychiatry in India. *International Journal of Health Services*, 38(3), 561–584. <https://doi.org/10.2190/HS.38.3.i>; Shidhaye, R., Shrivastava, S., Murhar, V., Samudre, S., Ahuja, S., Ramaswamy, R., & Patel, V. (2016). Development and piloting of a plan for integrating mental health in primary care in Sehore district, Madhya Pradesh, India. *British Journal of Psychiatry*, 208(s56), s13–s20. <https://doi.org/10.1192/bjp.bp.114.153700>

³⁴ Ministry of Health and Family Welfare, Government of India. (2024, October 13). *Tele MANAS: Revolutionizing mental health care in India* [Press release]. Press Information Bureau <https://www.pib.gov.in/PressNoteDetails.aspx?NotelD=153277&ModuleId=3®=3&lang=2>

³⁵ Sagar, R., & Singh, S. (2022). National Tele-Mental Health Program in India: A step towards mental health care for all?. *Indian journal of psychiatry*, 64(2), 117–119.

Bharat efforts to embed mental health within comprehensive primary care at Health and Wellness Centres.³⁶ Independent commentaries highlight unresolved concerns about quality, continuity and equity, noting that tele-platforms are constrained in their ability to deliver complex psychosocial interventions, that state-wise variation in staffing and referral networks leads to uneven service quality and that populations without phone access or digital literacy may be further marginalised.³⁷

Taken together, the literature suggests that India is in a transitional phase between an institutional, hospital-dominated model and a more distributed, community-anchored system of care. Policy frameworks such as NMHP 2014 and subsequent programme revisions articulate a clear preference for integrated, rights-oriented, community-based services, and recent expansions of DMHP, GHPUs and Tele-MANAS demonstrate movement in that direction. Persistent under-funding, human-resource shortages, weak inter-sectoral coordination and the absence of robust community-based rehabilitation and housing options, however, mean that many service users still experience public mental health care as medication-centric outpatient encounters or prolonged stays in overcrowded institutions rather than as continuous, holistic support within their own communities. For advocacy, this evidence underscores the importance of redirecting investment from custodial beds toward fully staffed district mental health teams, integrated primary-care and community services, peer-led and psychosocial programmes and rights-based governance mechanisms co-designed with persons who have lived experience of psychosocial disability.

3.3 Reports on experiences and on quality of care

a. Institutional Conditions and Material Quality of Care

The material condition of mental health institutes is central to determining the quality of care provided here. Material condition here entails physical infrastructure, availability and the quality of basic facilities such as beds, food, clean water, clothing, lavatory, separate wards, etc., staff configurations and the utilisation of central and state budget allocated towards this. Decent material condition is one of the minimum requirements needed to ensure the inhabitants a life of dignity and safety. The lack of this could exacerbate distress, increase risk

³⁶ Sagar, R., & Singh, S. (2022). National Tele-Mental Health Program in India: A step towards mental health care for all?. *Indian journal of psychiatry*, 64(2), 117-119.

³⁷ Dua, K. (2024). Attitude, Knowledge and Perception Among General Public About Tele-Manas Services in Delhi NCR. *International Journal of Preventive Medicine and Health (IJPMH)*, 4(5), 1-11.; Haenssger, M. J. (2018). The struggle for digital inclusion: Phones, healthcare, and marginalisation in rural India. *World Development*, 104, 358-374.

of harm, and undermine recovery. Article 28 of the CRPD requires that people with disabilities are provided with an adequate standard of living and assistance needed for continuous improvement of their living conditions. The following section thus presents an overview of what constitutes the contemporary (2000-present) materiality of living within mental health institutes in India.

The physical infrastructure of public mental health institutions has been persistently deficient in India. They frequently operate in antiquated structures that were originally designed as asylums during the colonial era following custodial design. Many of them have not systematically revamped to meet contemporary standards of therapeutic care. Performance audits and fact-finding reports describe overcrowded wards, deteriorating structures, inadequate ventilation, insufficient sanitation facilities and poor maintenance of living spaces. In several institutions, bed capacity falls well below patient loads, resulting in patients sleeping on floors or in congested dormitories with minimal privacy. Such conditions are not confined to a single state but recur across diverse contexts, suggesting structural rather than incidental shortcomings in institutional planning and investment.^{38 39 40}

The absence of differentiated and specialised infrastructure further constrains the quality of care. Reviews of state mental health facilities consistently note the lack of dedicated wards for women, older persons, children and adolescents, persons with substance-use disorders or those requiring intensive or short-term observation.⁴¹

Human resource constraints intersect closely with material conditions to shape everyday care environments. State audits and programme reviews repeatedly highlight severe shortages of psychiatrists, clinical psychologists, psychiatric social workers, nurses and trained support staff in public mental health institutions. In some states, vacancy rates for key cadres exceed half of sanctioned posts, with particularly acute deficits in non-medical mental health professionals.⁴² These shortages have direct implications for institutional life: reduced staff-to-patient ratios limit supervision and engagement, contribute to custodial routines

³⁸ Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

³⁹ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

⁴⁰ Audit Reports | Comptroller and Auditor General of India. (n.d.). <https://cag.gov.in/en/audit-report/details/5760>

⁴¹ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

⁴² Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

oriented around surveillance and control, and curtail the provision of psychosocial, rehabilitative and recovery-oriented interventions. Where staffing is thin, institutions often prioritise basic order maintenance and medication distribution over therapeutic interaction, reinforcing biomedical and custodial modes of care.

Living conditions within institutions which encompasses sanitation, nutrition, clothing, personal space and access to basic amenities, emerge as another critical dimension of material quality. Qualitative studies and inspection reports describe inadequate and poorly maintained toilet facilities, limited access to clean water, insufficient or monotonous food and shortages of clothing, bedding and menstrual hygiene supplies. Privacy is minimal in large dormitory-style wards, with personal belongings often restricted or inaccessible. Such conditions have been shown to exacerbate distress, undermine dignity and negatively affect both mental and physical health, particularly for individuals with histories of trauma, violence or social marginalisation. In mental health contexts, where feelings of vulnerability and loss of control are already pronounced, degraded living environments can intensify symptoms and impede recovery.⁴³

The literature also draws attention to the neglect of physical healthcare within psychiatric institutions as a material quality-of-care issue. Many public mental hospitals lack on-site facilities, equipment or trained personnel to manage common medical comorbidities associated with severe mental illness, including metabolic disorders, cardiovascular disease and infectious conditions. Routine monitoring of physical health parameters is often absent, and referral pathways to general medical services are weak. In overcrowded and unsanitary environments, these gaps increase the risk of preventable morbidity and mortality. Intensive care units (ICU) equipped with emergency drugs, oxygen, suction facilities, etc. are found to be often absent or grossly inadequate. These institutions are severely ill-equipped to handle any critical emergency situations.⁴⁴

In conclusion, material deprivation within public mental health institutions does more than signal resource scarcity; it actively structures the forms of care that become possible. Overcrowded wards, understaffed units and inadequate facilities create environments in

⁴³ School of Law Rights and Constitutional Governance -TISS, Mumbai & Maharashtra State Human Rights Commission. (2022). FACT-FINDING REPORT PROMOTING, MONITORING, AND UPHOLDING HUMAN RIGHTS FOR MARGINALISED SECTIONS OF SOCIETY IN MAHARASHTRA (By Maharashtra State Human Rights Commission & School of Law Rights and Constitutional Governance -TISS, Mumbai).

⁴⁴ Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

which containment, standardisation and risk avoidance are prioritised over engagement, autonomy and recovery. In such contexts, rights-based commitments articulated in national policy and legislation are difficult to realise in practice, as the institutional environment itself constrains consent, participation and therapeutic choice. Understanding institutional conditions as a central component of care quality is essential to any analysis of India's public mental health system and provides the necessary foundation for examining care practices, treatment regimes and experiences of autonomy in subsequent sections. Institutional materiality such as overcrowding, staff shortages and inadequate diagnostic infrastructure, shape the kind of environment in which pharmacological management becomes the most feasible mode of intervention. Hence, the materially constrained setting fundamentally structures clinical governance.

b. Care Practices, Treatment Regimes, and Clinical Governance

The literature on public mental health institutions in India indicates that care practices and treatment regimes are shaped by a combination of biomedical orientation, resource constraints, and weak clinical governance structures. Across states, inpatient and outpatient care within public mental health facilities is predominantly organised around pharmacological treatment, with psychotropic medication constituting the primary therapeutic intervention. Technical Committee reviews report that a majority of institutions indicate satisfactory availability of medication (77%), and that electroconvulsive therapy (ECT) services are offered in most hospitals (approximately 70%), with nearly all facilities reporting the use of modified ECT. These findings suggest that biomedical interventions are institutionally prioritised and operationally routinised within public mental hospitals, even as other dimensions of care remain underdeveloped.⁴⁵

At the same time, audit findings point to significant gaps in the clinical infrastructure required to safely and effectively support such treatment regimes. High rates of medical co-morbidity are reported in at least one-third of hospitals, yet diagnostic and monitoring facilities remain uneven and, in some cases, entirely absent. While national minimum standards issued by the National Institute of Mental Health and Neuro Sciences (NIMHANS) prescribe the availability of routine blood and urine investigations, lithium estimations, X-ray, ECG and EEG facilities within mental hospitals, Gujarat's audit reveals substantial non-compliance. Diagnostic laboratories were unavailable in multiple hospitals, requiring samples to be sent externally or patients to be referred to general medical colleges, raising concerns about delays and the

⁴⁵ Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

management of medical emergencies.⁴⁶ The literature thus highlights a structural mismatch between the intensity of biomedical treatment delivered and the institutional capacity to monitor physical health and treatment side-effects, particularly for individuals receiving long-term psychotropic medication or ECT.

Shortages of mental health professionals further shape treatment practices and constrain the scope of care. Performance audits of Gujarat observe acute deficits across all cadres in Hospitals for Mental Health (HMHs) and District Mental Health Programme (DMHP) units. Among sanctioned posts, approximately 62 per cent of psychiatrist positions, 50 per cent of psychologist posts and 56 per cent of psychiatric social worker posts remain vacant in HMHs, with some institutions functioning for extended periods without any psychiatrist or clinical psychologist. These shortages have direct implications for clinical processes: audits note that standard protocols for social, psychological, clinical and psychiatric assessment were not followed in most hospitals, with compliance reported only in isolated facilities. In district settings, the absence of psychiatrists and clinical psychologists in multiple DMHP units resulted in the unavailability of inpatient services and limited outpatient care, effectively reducing treatment to periodic consultations and medication dispensing.⁴⁷

Within this context, psychosocial and rehabilitative components of care are systematically marginalised. Observations from institutional assessments indicate that occupational therapy and rehabilitation centres, where present, are frequently under-utilised or non-functional, with equipment left unattended, rusted or damaged over long periods. Despite formal inclusion of rehabilitation within institutional mandates and programme guidelines, the lack of trained personnel, poor maintenance and absence of monitoring mechanisms render such facilities largely symbolic. As a result, care practices remain focused on symptom management rather than functional recovery, social reintegration or skill development, reinforcing a narrow clinical model that prioritises stabilisation over long-term outcomes.^{48 49}

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⁴⁶ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

⁴⁷ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

⁴⁸ Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

⁴⁹ School of Law Rights and Constitutional Governance -TISS, Mumbai & Maharashtra State Human Rights Commission. (2022). FACT-FINDING REPORT PROMOTING, MONITORING, AND UPHOLDING HUMAN RIGHTS FOR MARGINALISED SECTIONS OF SOCIETY IN MAHARASHTRA (By Maharashtra State Human Rights Commission & School of Law Rights and Constitutional Governance -TISS, Mumbai).

⁵⁰ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

Training and capacity-building mechanisms intended to support quality care also appear poorly implemented. DMHP guidelines provide for structured training of medical officers and paramedical staff at Centres of Excellence and district hospitals, as well as sensitisation programmes for community health workers and elected representatives. However, audits report that training durations were often limited to a single day when required to run for three days, with course content inadequately covered. In several places no training programmes were organised at all. The absence of systematic training for primary- and secondary-level health personnel undermines early detection, continuity of care and appropriate referral, while also reinforcing dependence on specialist-driven, hospital-centred treatment models.⁵¹

In conclusion, the literature depicts a system in which care practices are heavily medicalised, unevenly governed and weakly regulated. While medication availability and certain biomedical interventions are institutionalised, deficits in staffing, diagnostics, rehabilitation and training constrain the delivery of comprehensive, person-centred care. Clinical governance structures which are shaped by leadership gaps, inadequate monitoring and limited capacity-building, allow these patterns to persist, normalising treatment regimes that emphasise control and symptom suppression over recovery, participation and continuity. These systemic features provide the immediate context for examining how consent, coercion and autonomy are experienced within public mental health institutions, and how institutional care practices intersect with broader questions of rights and dignity. Hence, it can be seen how practices of coercion and constrained consent is not incidental, it is rather structurally embedded. Thus, these patterns of care have implications that extend beyond treatment outcomes. It shapes how authority is exercised, how decisions are made, and how patients experience consent and choice within institutional settings.

c. Experiences of Coercion, Consent, and Loss of Autonomy

A substantial body of Indian mental health literature highlights coercion, constrained consent, and diminished autonomy as persistent features of care within public mental health institutions, particularly for individuals admitted involuntarily or retained for long periods. While policy frameworks and the Mental Healthcare Act emphasise on having informed consent, least-restrictive care and community living, empirical studies, audits and rehabilitation reviews suggest that these principles are unevenly realised in practice. Instead, institutional routines, family involvement, clinical authority and the absence of viable

⁵¹ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

community alternatives frequently combine to produce situations in which individuals experience mental healthcare as imposed rather than chosen.

Long-stay institutionalisation is one of the most common ways in which autonomy is curtailed. Reviews and technical committee reports note the continued presence of substantial long-stay populations in state mental hospitals, particularly in Maharashtra, Punjab, Tamil Nadu and West Bengal.⁵² Rehabilitation initiatives consistently document the complexity of discharge and reintegration, especially for individuals who come from poverty, lack family support or have spent extended periods in institutions. Experiences from Gujarat indicate that earlier rehabilitation interventions and sustained collaboration with community agencies are associated with better outcomes, while delayed or abrupt efforts to relocate long-stay patients can generate new forms of vulnerability. Longer-term follow-up from institutions such as the Lokopriya Gopinath Bordoloi Regional Institute of Mental Health underscores the need for personalised, phased rehabilitation strategies rather than en masse discharge orders, which even though well-intentioned may have unforeseen adverse consequences, including homelessness, relapse and social isolation.⁵³

Audit findings further demonstrate how the absence of rehabilitation infrastructure contributes to prolonged institutionalisation and involuntary retention. Rehabilitation is widely recognised as a core component of mental healthcare, intended to address long-standing disability and support the development of social, cognitive and vocational skills necessary for community integration. National guidance envisages a continuum of facilities, including long-stay homes for individuals requiring ongoing support and halfway homes for those transitioning out of hospital care. However many of those who have been deemed clinically fit for discharge for periods ranging from one to five years remained in hospitals because state governments had not established rehabilitation centres, either independently or in collaboration with non-governmental organisations. In the absence of such facilities, institutional care effectively becomes indefinite, converting what should be therapeutic admission into prolonged custodial confinement.

The erosion of autonomy is also evident in everyday admission and treatment experiences. Empirical studies examining patient perspectives on psychiatric admission reveal complex patterns of formal and informal coercion. While a majority of participants report that they

⁵² Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

⁵³ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

were not physically forced, threatened or physically compelled to enter hospital, a substantial proportion indicate that they were threatened with involuntary commitment, suggesting the presence of indirect or symbolic coercion. Coercion in this context is conceptualised not only as overt force but as the subjective experience of loss of autonomy arising from involuntary or pressured interactions with mental health services.⁵⁴

Patients' affective responses to hospitalisation further illuminate the experiential dimensions of constrained consent. Studies report that most participants described feeling sad, unpleasant, confused and frightened during admission, with fewer reporting relief or reassurance. These emotional responses were more pronounced among individuals admitted involuntarily, who perceived significantly higher levels of formal coercion compared to those who entered hospital voluntarily. At the same time, even patients classified as voluntary admissions reported experiencing informal coercion, such as negative pressure from family members or implicit threats of compulsory admission. Together, these findings suggest that the binary distinction between voluntary and involuntary admission obscures a broader continuum of coercive experiences embedded within institutional and familial decision-making processes.⁵⁵

The literature thus indicates that loss of autonomy in public mental health institutions is not solely the result of individual clinical decisions but is structurally produced through long-stay institutionalisation, inadequate rehabilitation infrastructure, weak community linkages and risk-averse governance practices. Coercion operates along a spectrum, ranging from formal involuntary admission to subtler forms of pressure and constraint embedded in institutional routines and family dynamics. These dynamics are particularly pronounced for individuals who come from poverty, socially marginalised or disconnected from family support, for whom institutional care may become the only available form of shelter and assistance. Understanding coercion and constrained consent as systemic phenomena rather than exceptional deviations is therefore essential for evaluating the rights-compatibility of public mental health care and for informing reforms aimed at restoring autonomy, choice and dignity.

⁵⁴ Poreddi, V., Ramu, R., Selvi, S., Gandhi, S., Krishnasamy, L., & Suresh, B. M. (2016). Admission experiences of psychiatric patients in tertiary care: An implication toward Mental Health Care Bill, 2013. *Journal of Neurosciences in Rural Practice*, 08(01), 089–095. <https://doi.org/10.4103/0976-3147.193527>

⁵⁵ Poreddi, V., Ramu, R., Selvi, S., Gandhi, S., Krishnasamy, L., & Suresh, B. M. (2016). Admission experiences of psychiatric patients in tertiary care: An implication toward Mental Health Care Bill, 2013. *Journal of Neurosciences in Rural Practice*, 08(01), 089–095. <https://doi.org/10.4103/0976-3147.193527>

d. Gendered and Socially Differentiated Experiences of Care

The literature consistently demonstrates that experiences of care within public mental health institutions in India are deeply gendered and socially differentiated, shaped by intersecting hierarchies of gender, caste, class, marital status, family relations and economic dependence. Women, in particular, encounter mental health institutions not only as sites of clinical intervention but also as extensions of broader patriarchal and exclusionary social arrangements. Reviews, inspections and qualitative studies converge in showing that institutional care often reproduces gendered vulnerabilities, with women disproportionately represented among long-stay, abandoned and destitute patients.

National-level inspections conducted by the National Commission for Women (NCW) reveal systematic gender-specific deficiencies in public psychiatric institutions, particularly for women admitted as inpatients. Across 27 government psychiatric homes reviewed and 19 institutions physically inspected, the Commission documented persistent gaps in infrastructure, staffing and governance that directly compromised women's dignity, privacy and safety.⁵⁶ Female wards were frequently overcrowded, inadequately segregated or, in some cases, entirely absent, with several institutions lacking regulated entry and exit controls for women's spaces. Privacy during bathing, changing clothes and toileting was routinely compromised, while access to basic gender-sensitive necessities such as personal toiletries, sanitary napkins, undergarments and individual clothing was inconsistent and poorly standardised. These material deprivations were compounded by shortages of female staff, including nurses and psychiatric social workers, limiting women's ability to seek care, report abuse or participate meaningfully in treatment decisions.

Family abandonment and consequent prolonged institutionalisation emerge as central themes in women's experiences of care. The NCW documents that women's inpatient stays are often significantly longer than those of men, with many women remaining institutionalised for more than five years after clinical stabilisation. Inspections across institutions in Indore, Jaipur, Amritsar, Bareilly and Kolkata identified large numbers of women who were effectively "over-staying" due to family rejection, marital breakdown, property disputes or remarriage of spouses. In contrast to male patients, whose families were more likely to facilitate discharge and reintegration, women were frequently described as unwanted burdens, particularly when they were unmarried, widowed, divorced or unable to

⁵⁶ The People's Archive of Rural India. (n.d.). Review of Psychiatric Homes/Mental Hospitals of Government Sector in India with special reference to the female patients in IPD. People's Archive of Rural India. <https://ruralindiaonline.org/en/library/resource/review-of-psychiatric-homesmental-hospitals-of-government-sector-in-india-with-special-reference-to-the-female-patients-in-ipd/>

resume expected domestic roles. The absence of legal or policy mechanisms compelling families to assume responsibility, combined with weak institutional efforts to trace relatives or provide family counselling, resulted in women becoming indefinitely institutionalised.^{57 58}

Qualitative feminist scholarship provides deeper insight into how these institutional patterns are experienced and internalised by women themselves. Bhattacharya's narrative study of women with serious mental illness foregrounds how poverty, gender-based violence, restricted life opportunities and lifelong dependence on family structures shape pathways into psychiatric institutions and limit possibilities for exit.⁵⁹ Women's narratives reveal that admission to mental hospitals often followed experiences of domestic violence, abandonment by spouses, or rejection by natal families, rather than acute psychiatric crises alone. Institutionalisation, in this sense, functioned as a form of social containment for women whose distress disrupted normative gender roles. Even transitions to less restrictive settings such as halfway homes did not necessarily restore autonomy; instead, women described these spaces as extensions of institutional control, marked by surveillance, moral regulation and continued dependence.

The gendered nature of care is further evident in how treatment, consent and participation are structured within institutions. NCW findings highlight widespread dissatisfaction among female patients regarding non-participation in healthcare decisions, lack of information about diagnoses and medication, and the absence of consent in routine practices such as haircuts or personal grooming. Standard operating procedures sensitive to women's needs were largely absent, and confidentiality around women's treatment and personal histories was frequently breached. These practices reinforce a paternalistic model of care in which women's voices are marginalised and their agency systematically undermined.

Social differentiation along lines of caste, class and economic dependence further compounds gendered disadvantage. Women from lower classes, Dalit and tribal communities, and those without formal identification documents were more likely to be admitted involuntarily, remain untraceable to families and lack access to legal aid or

⁵⁷ The People's Archive of Rural India. (n.d.). *Review of Psychiatric Homes/Mental Hospitals of Government Sector in India with special reference to the female patients in IPD*. People's Archive of Rural India. <https://ruralindiaonline.org/en/library/resource/review-of-psychiatric-homesmental-hospitals-of-government-sector-in-india-with-special-reference-to-the-female-patients-in-ipd/>

⁵⁸ Tr, R. (2025). Family and Societal attitudes toward Women with Mental Illness in India: A Systematic Review of Barriers and Support. *International Journal of Medical and All Body Health Research*, 6(2), 28-37. 10.54660/IJMBHR.2025.6.2.28-37

⁵⁹ Bhattacharya, A. (2019). Women's Narratives on Illness and Institutionalization in India: A Feminist Inquiry. Columbia University Libraries. <https://doi.org/10.7916/d8-f7rs-3p58>

rehabilitation services. Bhattacharya's analysis underscores how poverty and gender intersect to render women "easy to admit but difficult to discharge," as institutional care becomes a substitute for absent social protection systems. The absence of halfway homes, community housing and livelihood-linked rehabilitation disproportionately affects women, for whom independent living is rarely considered a viable option within prevailing social norms .

In conclusion, the literature illustrates that public mental health institutions, like many other institutions in India, operate within patriarchy, often reinforcing gendered and socially stratified systems of exclusion. Women's experiences of care or lack thereof are shaped by their position within family structures, labour markets and social hierarchies. Institutional practices that neglect gender-sensitive infrastructure, rehabilitation, legal safeguards and community reintegration disproportionately affect women, transforming therapeutic spaces into sites of prolonged social abandonment. Addressing these inequities requires moving beyond gender-neutral models of reform toward explicitly feminist, intersectional and rights-based approaches that recognise women's lived realities and centre their agency within mental healthcare systems.

3.3 Implementation Gaps: Identifying Faultlines

Implementation gaps in India's public mental health system are consistently highlighted across policy reviews, scholarly analyses, CRPD reporting and judicial proceedings. These gaps appear not only as technical shortfalls but as indicators of deeper structural and political constraints that limit the realisation of rights-based, community-oriented care envisioned in the National Mental Health Policy 2014 and the Mental Healthcare Act 2017.

a. Lack of Transparency & Data Gaps

Reviews of the National Mental Health Policy 2014 emphasise the absence of a robust national mental-health information system, noting that routine data on service utilisation, human resources, quality of care and rights violations are either not collected or reported in fragmented, non-standardised formats.⁶⁰ India's report to the CRPD Committee similarly

⁶⁰ Gupta, S., & Sagar, R. (2022). National Mental Health Policy, India (2014): Where have we reached? *Indian Journal of Psychological Medicine*, 14(1), 1–6. <https://doi.org/10.1177/02537176211048335>; Gangadhar, B. N., Naveen Kumar, C., Sadh, K., Manjunatha, N., Math, S. B., Kalaivanan, R. C., ... & Chaturvedi, S. K. (2023). Mental health programme in India: Has the tide really turned? *Indian Journal of Medical Research*, 157(5), 387–394. https://doi.org/10.4103/ijmr.IJMR_2217_20; Ahmed, T., Dumka, N., Rizvi, S. M. A., Rizvi, S. N., & Rizvi, S. A. A. (2022). Understanding India's response to mental health care: A systematic review of the literature and overview of

acknowledges that data on institutionalisation, coercive practices and community-based services are partial and often derived from ad hoc surveys or programme-specific assessments rather than a comprehensive surveillance framework.⁶¹ Evaluations of the DMHP conclude that publicly available information on district coverage, service packages and performance is patchy and sometimes inconsistent even across government documents, which makes external assessment of progress extremely difficult.⁶²

Scholars argue that this data scarcity is itself evidence of systemic weakness because it prevents accurate estimation of the treatment gap, obscures patterns of long-stay institutionalisation and abuse and undermines rational planning of services.⁶³ Gupta and Sagar's review of NMHP 2014 identifies the lack of reliable monitoring data as a central reason why community rehabilitation and integration remain "abysmally low" despite clear policy commitments.⁶⁴ A recent systematic review of India's mental-health response similarly concludes that the paucity of routine data on coverage, quality and rights outcomes is a major obstacle to evaluating NMHP and DMHP and to using evidence for course correction.⁶⁵ From a human-rights perspective, the near invisibility of people in institutions, homeless populations and marginalised caste, class, gender and age groups within official statistics is interpreted as a form of structural erasure that weakens accountability and enables ongoing rights violations.

the National Mental Health Programme. *Journal of Global Health Neurology and Psychiatry*. <https://joghnp.scholasticahq.com/article/36128>

⁶¹ United Nations Committee on the Rights of Persons with Disabilities. (2019). *Concluding observations on the initial report of India* (CRPD/C/IND/CO/1). <https://cdnbbsr.s3waas.gov.in/s3e58aea67b01fa747687f038dfde066f6/uploads/2023/09/20230927301902296.pdf>

⁶² Centre for Mental Health Law & Policy. (2021). *Deconstructing the DMHP: Part IV* [Issue brief]. <https://cmhlp.org/wp-content/uploads/2021/08/Issue-Brief-DMHP-IV.pdf>; Gangadhar, B. N., Naveen Kumar, C., Sadh, K., Manjunatha, N., Math, S. B., Kalaivanan, R. C., ... & Chaturvedi, S. K. (2023). Mental health programme in India: Has the tide really turned? *Indian Journal of Medical Research*, 157(5), 387–394. https://doi.org/10.4103/ijmr.IJMR_2217_20; Kirpekar, V. L., Nagra, V., & Jadhav, S. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(Suppl 2), S346–S353.

⁶³ Debnath, A., Sagar, R., & Salve, H. R. (2025). Bridging the mental health treatment gap in India: A policy-oriented framework using the care cascade approach. *Indian Journal of Psychiatry*, 67(9), 912–915. https://doi.org/10.4103/indianjpsychiatry.640_25; Suhas, S., Arvind, B. A., Sukumar, G. M., Banandur, P. S., Nirisha, L. P., Kumar, C. N., ... & Gururaj, G. (2023). A bird's eye view of the mental health systems in India. *Indian Journal of Psychiatry*, 65(12), 1214–1222. https://doi.org/10.4103/indianjpsychiatry.indianjpsychiatry_845_23

⁶⁴ Gupta, S., & Sagar, R. (2022). National Mental Health Policy, India (2014): Where have we reached? *Indian Journal of Psychological Medicine*, 14(5), 456–463. <https://doi.org/10.1177/02537176211048335>

⁶⁵ Ahmed, T., Dumka, N., Rizvi, S. M. A., Rizvi, S. N., & Rizvi, S. A. A. (2022). Understanding India's response to mental health care: A systematic review of the literature and overview of the National Mental Health Programme. *Journal of Global Health Neurology and Psychiatry*. <https://joghnp.scholasticahq.com/article/36128>

b. Administrative Hurdles to Stricter Implementation

Legal and policy analyses underline a persistent gap between the progressive text of MHCA 2017 and the subordinate legislation and administrative action required to operationalise it.⁶⁶ Commentators document substantial delays by several states in notifying MHCA rules, constituting State Mental Health Authorities and Mental Health Review Boards and registering mental health establishments, leaving many of the Act's rights and safeguards dormant years after commencement.⁶⁷ In **Dr Harish Shetty v. State of Maharashtra**, the Bombay High Court criticised the state for non-implementation of core MHCA obligations, including the failure to fully establish regulatory bodies and notify detailed norms, illustrating how legislative promises can remain purely aspirational without timely executive follow-through.⁶⁸ The literature on NMHP and DMHP notes similar problems with programme-level guidance, describing national guidelines as broad and ambitious but often vague on staffing norms, training, supervision and intersectoral coordination mechanisms that would enable districts to deliver the envisaged package of services.⁶⁹ Gupta and Sagar point out that NMHP 2014 itself does not specify independent review mechanisms for involuntary admissions or detailed safeguards for research, which limits its human-rights robustness even at the normative level.⁷⁰ Recent narrative reviews of NMHP implementation observe that, in the absence of clear, binding standards, many DMHP sites have defaulted to a narrow, medication-centred model with minimal psychosocial and community-engagement components. These deficits in rules and detailed standards create "implementation vacuums" in which frontline providers revert to older custodial or biomedical practices rather than the

⁶⁶ Punjab & Haryana High Court. (2025, May 20). *Mental Health Care Act, 2017: HC tells Punjab, Haryana to notify rules within sixty days*. Hindustan Times.

<https://www.hindustantimes.com/cities/chandigarh-news/mental-health-care-act-2017-hc-tells-punjab-haryana-to-notify-rules-within-sixty-days-101747123456789.html>

⁶⁷ Centre for Mental Health Law & Policy. (2025, November 10). *Tracking the Mental Healthcare Act, 2017 – 7 years on*. <https://cmhlp.org/imho/blog/tracking-the-mental-healthcare-act-2017-7-years-on/>;

⁶⁸ Bar & Bench. (2022, December 3). *"Regrettable state of affairs": Bombay High Court on failure of Maharashtra to implement Mental Healthcare Act*.

<https://www.barandbench.com/news/litigation/regrettable-bombay-high-court-failure-maharashtra-implement-mental-healthcare-act>

⁶⁹ Grover, S., Dua, D., Chakrabarti, S., Avasthi, A., & Kumar, S. (2018). National Mental Health Programme—Optimism and caution: A narrative review. *Indian Journal of Psychiatry*, 60(Suppl 4), S486–S494. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_191_18; Sinha, S. K., & Kaur, J. (2011). National mental health programme: Manpower development and training for mental health care. *Indian Journal of Psychiatry*, 53(4), 299–301. <https://doi.org/10.4103/0019-5545.86811>; Centre for Mental Health Law & Policy. (2021). *Deconstructing the DMHP: Part IV* [Issue brief]. <https://cmhlp.org/wp-content/uploads/2021/08/Issue-Brief-DMHP-IV.pdf>

⁷⁰ Gupta, S., & Sagar, R. (2022). National Mental Health Policy, India (2014): Where have we reached? *Indian Journal of Psychological Medicine*, 14(5), 456–463. <https://doi.org/10.1177/02537176211048335>

supported-decision-making and community-based approaches implied by MHCA 2017 and NMHP 2014.⁷¹

c. Financial Feasibility & Budgetary Limitations

A central theme across reviews is the chronic under-financing of mental health within India's overall health spending. Analyses of the NMHP budget show that allocations have historically constituted a tiny proportion of central health expenditure and that even these modest amounts often remain under-utilised, particularly for central initiatives such as Tele-MANAS, where utilisation has been well below budget estimates. Utilisation dropped to 0% in 2019-20 from over 100% in 2017-18 for centrally sponsored schemes including NMHP. Tele-MANAS saw FY 2023-24 allocation of ₹134 crore revised to just ₹65 crore (48% utilisation), despite rising call volumes. District Mental Health Programme (DMHP) funds showed wide variance: highs of 78% in Andhra Pradesh, lows of 5% in Telangana.⁷² Gupta and Sagar's appraisal of NMHP 2014 concludes that DMHP expansion has outpaced investment in human resources and infrastructure, resulting in district teams that are understaffed, reliant on short-term contractual workers and unable to provide the full spectrum of psychosocial and rehabilitative services envisioned in policy.⁷³ Cost-estimation work on the MHCA 2017 shows that full implementation of the Act, including access to essential psychotropic medicines, community services and minimum staffing norms, would require very substantial new public investment, estimated at over ₹94,000 crore annually, far above current spending levels which is less than 1% of the total healthcare budget. Expenditure for NMHP was roughly ₹5 crore annually in FY18 and FY19, despite MHCA 2017 needs estimated at ₹94,073 crore.⁷⁴ The authors warn that without such proactive resource mobilisation, the rights created by MHCA

⁷¹ Rao, T. S. S., Thippeswamy, H., & Narayana, M. (2023). Exploring psychiatrists' experiences during transition from Mental Health Act, 1987 to Mental Healthcare Act, 2017: A qualitative study. *Indian Journal of Psychiatry*, 65(10), 1053–1060. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_306_23; Centre for Mental Health Law & Policy. (2021). *Deconstructing the DMHP: Part IV* [Issue brief]. <https://cmhlp.org/wp-content/uploads/2021/08/Issue-Brief-DMHP-IV.pdf>; TKirpekar, V., Faye, A., Bhawe, S., Gawande, S., & Tadke, R. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(7), 603–613.

⁷² Mahashur, S. (2024, March 7). *India Mental Health Budget 2024- 2025 allocations*. CMHLP. <https://cmhlp.org/imho/blog/the-interim-union-budget-fy-2024-25-where-does-mental-health-stand>; Ahmed, T., Dumka, N., Hannah, E., Chauhan, V., & Kotwal, A. (2022). Understanding India's response to mental health care: a systematic review of the literature and overview of the National Mental Health Programme. *Journal of Global Health Neurology and Psychiatry*, e2022010.

⁷³ Gupta, S., & Sagar, R. (2022). National mental health policy, India (2014): where have we reached?. *Indian Journal of Psychological Medicine*, 44(5), 510–515.

⁷⁴ Math, S. B., Gowda, G. S., Basavaraju, V., Manjunatha, N., Kumar, C. N., Enara, A., Gowda, M., & Thirthalli, J. (2019). Cost estimation for the implementation of the Mental Healthcare Act 2017. *Indian journal of psychiatry*, 61(Suppl 4), S650–S659. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_188_19

2017 “will remain aspirational,” a conclusion echoed by broader programme reviews that link limited budgetary commitment to the persistence of large treatment gaps and the slow pace of de-institutionalisation. A 2023 narrative review of the Mental Health Programme in India notes that, four decades after NMHP was launched, services remain “inadequate, inequitably distributed and poorly integrated,” and explicitly attributes much of this stagnation to low, unstable and poorly targeted financing for NMHP and DMHP.⁷⁵ Recent civil-society budget analyses also show under-utilisation of Tele-MANAS allocations and continued dominance of spending on central institutions compared with community-based services, reinforcing concerns about the feasibility of a community-oriented, rights-based model under current fiscal priorities.⁷⁶

d. Weak or Inadequate Monitoring Mechanisms

Under the MHCA 2017, regulatory and quasi-judicial bodies such as the Central and State Mental Health Authorities and Mental Health Review Boards are intended to set standards, register and inspect establishments and offer accessible complaint and review mechanisms. However, India’s report to the UNCRPD Committee, judicial findings in ***Dr Harish Shetty v. State of Maharashtra*** and independent legal analyses document delays in establishing these bodies, lack of dedicated staff and infrastructure and minimal public awareness of their role, leading to limited oversight of both public and private institutions. Scholarly reviews argue that in the absence of active regulators and review boards, practices such as prolonged involuntary admission, use of restraints and neglect of rehabilitation can continue largely unchecked, even though they conflict with both MHCA 2017 and UNCRPD standards.⁷⁷

Programme-level monitoring is also described as weak. Evaluations of NMHP and DMHP repeatedly highlight the lack of a robust monitoring and evaluation system, noting that available assessments are sporadic, methodologically heterogeneous and often donor- or

⁷⁵ Gangadhar, B. N., Naveen Kumar, C., Sadh, K., Manjunatha, N., Math, S. B., Kalaivanan, R. C., ... & Chaturvedi, S. K. (2023). Mental health programme in India: Has the tide really turned? *Indian Journal of Medical Research*, 157(5), 387–394. https://doi.org/10.4103/ijmr.IJMR_2217_20

⁷⁶ Centre for Mental Health Law & Policy. (2025, March). *Union Budget for Mental Health 2025–2026* (IMHO Brief). <https://cmhlp.org/wp-content/uploads/2025/03/IMHO-Budget-For-Mental-Health-2025–2026.pdf>; Mahashur, S., & Pillutla, R. (2025, August 7). *Mental health funding in India: When economic surveys and budget realities diverge*. The Wire. <https://thewire.in/health/mental-health-funding-in-india-when-economic-surveys-and-budget-realities-diverge>

project-driven rather than embedded in routine governance structures.⁷⁸ A 2024 review points to administrative delays, incomplete analysis of DMHP data and absence of clear linkages with broader health reforms, and concludes that many districts have reduced DMHP activities to outpatient clinics and drug dispensing, with school programmes, community outreach and rehabilitation receiving far less attention than envisaged.⁷⁹ A 2022 systematic review of India's mental-health response similarly finds that limited community participation, minimal involvement of users and carers in oversight and weak engagement of national human-rights institutions undermine accountability and slow the translation of legal and policy advances into everyday practice.⁸⁰

4. Alternative Practice and Models

Critiques of custodial and hospital-centred psychiatric care in India have, over several decades, catalysed the development of alternative models of mental healthcare rooted in community engagement, psychosocial support and rights-based practice. These approaches have emerged not only in response to documented human rights violations and poor quality of care within mental health institutions, but also as pragmatic responses to India's persistent treatment gap, workforce shortages and the social determinants of mental distress. Community-based care is increasingly recognised as both more humane and more effective than institution-centred models, particularly for people who are homeless, socially marginalised or living with long-term mental illness. (Thorncroft et al., 2016).⁸¹ Evidenced in initiatives such as The Banyan's Adaikalam programme in Tamil Nadu, which reorients care for homeless women with mental illness away from long-term hospitalisation towards

⁷⁸ Gupta, S., & Sagar, R. (2022). National Mental Health Policy, India (2014): Where have we reached? *Indian Journal of Psychological Medicine*, 14(1), 1–6. <https://doi.org/10.1177/02537176211048335>; Gangadhar, B. N., Naveen Kumar, C., Sadh, K., Manjunatha, N., Math, S. B., Kalaivanan, R. C., ... & Chaturvedi, S. K. (2023). Mental health programme in India: Has the tide really turned? *Indian Journal of Medical Research*, 157(5), 387–394. https://doi.org/10.4103/ijmr.IJMR_2217_20; Kirpekar, V. L., Nagra, V., & Jadhav, S. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(Suppl 2), S346–S353. https://doi.org/10.4103/psychiatry.IndianJPsychiatry_105_24

⁷⁹ Kirpekar, V., Faye, A., Bhawe, S., Gawande, S., & Tadke, R. (2024). District mental health program: Then and now. *Indian Journal of Psychiatry*, 66(7), 603–613.

⁸⁰ Ahmed, T., Dumka, N., Rizvi, S. M. A., Rizvi, S. N., & Rizvi, S. A. A. (2022). Understanding India's response to mental health care: A systematic review of the literature and overview of the National Mental Health Programme. *Journal of Global Health Neurology and Psychiatry*. <https://joghnp.scholasticahq.com/article/36128>

⁸¹ Uvais, N. A., & Joag, K. (2024). Perceptions regarding the Indian Mental Healthcare Act 2017 among psychiatrists: Review and critical appraisal in the light of CRPD guidelines. *Global Mental Health*, 11, e39. <https://doi.org/10.1017/gmh.2024.31>; Ghosh, S., Gupta, S., & Singh, V. (2022). Mental Healthcare Act, 2017 (MHCA), from paper to clinical practice in the mental health settings. *Indian Journal of Psychological Medicine*, 44(3), 315–316. <https://doi.org/10.1177/02537176221084045>

community living and psychosocial rehabilitation, as well as in hybrid arrangements like the Dava-Dua Initiative, where psychiatric services are delivered through culturally significant healing spaces rather than detached clinical institutions. Similarly, peer-led women's self-help groups operating at the community level have linked mental healthcare with livelihood support and social participation, demonstrating how recovery can be sustained through economic stability and collective care rather than custodial confinement. These initiatives powerfully highlight how mental healthcare can be effectively delivered outside closed institutions while still ensuring continuity, safety and long-term recovery.

At a conceptual level, these alternatives challenge the assumption that recovery is best achieved through prolonged institutionalisation or specialist-led biomedical treatment alone. Instead, they emphasise early intervention, continuity of care, social reintegration and the rebuilding of agency through sustained engagement with families, communities and peer networks. This shift is evident in models such as task-sharing programmes, where frontline workers like ASHAs are trained to identify and support common mental health conditions, as seen in Sangath's VISHRAM programme; mobile mental health units, which bring psychiatric consultation and follow-up to remote or underserved areas; and tele-mental health platforms, which enable ongoing care where specialist access is limited. As reflected in national policy discourse and WHO guidance, integrating mental health into primary care, community platforms and non-clinical settings, such as temple-based counselling or SHG meeting spaces, allows care to be delivered closer to people's everyday lives, reduces stigma associated with psychiatric hospitals and mitigates the risk of long-term institutionalisation. Psychosocial interventions centred on empowerment, resilience and dignity reframe mental healthcare as a relational and social process rather than a purely clinical one.

4.1 What Community Practitioners and Nonprofits Identify as Quality Care

Indian non-governmental organisations (NGOs) and practitioner collectives have been central to articulating what quality care looks like outside institutional settings. Organisations such as **The Banyan (Tamil Nadu)**, **Sangath (Goa)**, **Iswar Sankalpa (West Bengal)**, **Ashadeep (Assam)**, **Aashray Adhikar Abhiyan (Delhi)** and the **Richmond Fellowship Society (Karnataka)** have worked extensively with homeless persons with mental illness and long-stay institutional populations, developing models that integrate clinical care with housing, livelihoods and social support.⁸²

⁸² Dr Debanjan Banerjee. (2023, May 23). Vicious link between homelessness, mental health. The New Indian Express.
<https://www.newindianexpress.com/opinions/2023/May/17/vicious-link-between-homelessness-mental-health-2576066.html>

Across these initiatives, quality care is consistently defined in terms of trauma-informed practice, community-based delivery, psychosocial rehabilitation, peer and family engagement, and explicit commitments to dignity and autonomy. For example, *The Banyan's Adaikalam Programme* in Tamil Nadu has, since the early 1990s, rehabilitated over 3,000 homeless women with mental illness through a structured transitional housing model that combines psychiatric care with social work, livelihood support and long-term follow-up, reporting high rates of sustained community living.⁸³ Rather than treating homelessness and mental illness as separate problems, the model recognises the cumulative effects of gender-based violence, abandonment and poverty, and organises care around safety, trust and gradual restoration of agency.

Similarly, Sangath's VISHRAM and community mental health programmes demonstrate how task-sharing and community-based psychosocial interventions can address severe shortages of mental health professionals. By training ASHA workers and lay counsellors, Sangath has expanded access to maternal and common mental health care, contributing to documented reductions in perinatal suicide and improved treatment adherence.⁸⁴ These approaches foreground relational continuity and culturally sensitive engagement, in sharp contrast to episodic outpatient encounters or custodial inpatient care.

Peer networks and self-help groups emerge as another defining feature of quality care in community models. Women participating in mental health-focused SHGs demonstrate significantly higher treatment adherence and confidence in help-seeking, largely due to the combined effects of economic empowerment, stigma reduction and crisis support. Initiatives such as Maharashtra's Asha-Didi model, which integrates mental health education with vocational training, have achieved high employment and retention rates among women with psychosocial disabilities, reinforcing the link between economic security and mental health recovery.⁸⁵

⁸³ The Banyan. (2025, May 12). The Banyan | Mental Health & Social Care Support Services. <https://thebanyan.org/about-us/>

⁸⁴ Tata Trusts-supported Vishram to culminate in agricultural communities of Vidarbha - Press releases - Tata Trusts. (n.d.). Tata Trusts. <https://www.tatatrusts.org/media/press-releases/tata-trusts-supported-vishram-to-culminate-in-agricultural-communities-of-vidarbha>

⁸⁵ Tr, R. (2025). Family and Societal attitudes toward Women with Mental Illness in India: A Systematic Review of Barriers and Support. *International Journal of Medical and All Body Health Research*, 6(2), 28-37. 10.54660/IJMBHR.2025.6.2.28-37

4.2 Lessons from Community Alternatives: What Public Mental Health Institutions Are Missing

Taken together, these community-based initiatives illuminate what public mental health institutions systematically fail to provide. First, they foreground psychosocial rehabilitation as a core component of care rather than an optional add-on. Audits of public mental health institutions in Gujarat note that occupational therapy and rehabilitation units are under-utilised or non-functional.⁸⁶ NGO models, however, demonstrate that structured skill-building, supported employment and livelihood programmes can significantly improve functioning and long-term outcomes.

Second, community models underscore the importance of continuity and longitudinal engagement. Projects supported by Tata Trusts at Yerawada Mental Hospital (Pune) and LGBRIMH, Tezpur illustrate this point clearly. Of the 669 long-stay patients identified at Yerawada, 200 were enrolled in a structured rehabilitation project that combined clinical care with NGO-led social work and community linkage. At Tezpur, interventions with homeless persons led to family reunification in several cases, echoing similar outcomes documented by The Banyan in Tamil Nadu. However, these same projects also highlight the constraints faced by NGOs, including inability to manage physical health emergencies, lack of regular psychiatric medication supply, shortage of trained staff and financial limitations. These gaps point to the need for stronger public-civil society collaboration rather than ad hoc reliance on NGOs alone.⁸⁷

Third, community alternatives reveal the value of peer involvement and lived experience in reshaping care relationships. Models such as Gujarat's Mann-Mooli shops, run by women with lived experience of mental illness, combine affordable access to medication with peer counselling in non-stigmatizing neighbourhood spaces, reporting reductions in relapse and high customer retention.⁸⁸ These initiatives challenge the hierarchical provider-patient relationships that dominate institutional settings and demonstrate how autonomy and trust can be built into service delivery.

⁸⁶ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

⁸⁷ Murthy, P., Kumar, S., Desai, N., & Teja, B. K. (2015). Report of the Technical Committee on Mental Health [Constituted by the National Human Rights Commission to evaluate mental health services in India]. National Human Rights Commission.

⁸⁸ Tr, R. (2025). Family and Societal attitudes toward Women with Mental Illness in India: A Systematic Review of Barriers and Support. *International Journal of Medical and All Body Health Research*, 6(2), 28-37. 10.54660/IJMBHR.2025.6.2.28-37

Finally, culturally embedded and hybrid models show how care can be delivered without reinforcing stigma or coercion. The Dava-Dua initiative in Uttar Pradesh and Gujarat, which trains faith healers in basic psychiatric referral protocols, has reduced harmful practices while increasing appropriate referrals and medication adherence.⁸⁹⁹⁰ Similarly, the Gujarat government's agreement with an NGO to run psychiatric clinics at the Mira Datar Dargah illustrates how sacred spaces can function as entry points to mental healthcare, handling thousands of new and follow-up cases while remaining accessible to populations reluctant to approach formal hospitals.⁹¹

4.3 Developing an Expansive Conceptual Framework for Institutional Care

The evidence from community-based alternatives points toward the need for a more expansive conceptual framework for institutional mental healthcare in India. Rather than positioning hospitals and community care as mutually exclusive, these models suggest the importance of a continuum of care in which institutional settings play a limited, clearly defined and time-bound role. Public mental health institutions would function primarily as spaces for short-term stabilisation and crisis care, embedded within wider systems of community housing, rehabilitation, livelihood support and primary healthcare.

An expansive framework would require institutional care to be reorganised around trauma-informed principles, psychosocial rehabilitation and rights-based governance. This includes meaningful partnerships with NGOs, peer collectives and community agencies; integration of rehabilitation and livelihood planning into discharge processes; and mechanisms to prevent long-stay institutionalisation in the absence of community alternatives. Importantly, the challenges faced by NGOs, such as managing physical health needs or sustaining funding, underscore that community care cannot substitute for public investment, but must be supported through systemic integration and resource allocation.

In conclusion, these alternative practices do more than offer isolated success stories. They reveal, with specificity, what public mental health institutions are structurally missing:

⁸⁹ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

⁹⁰ Tr, R. (2025). Family and Societal attitudes toward Women with Mental Illness in India: A Systematic Review of Barriers and Support. International Journal of Medical and All Body Health Research, 6(2), 28-37. 10.54660/IJMBHR.2025.6.2.28-37

⁹¹ CAG of India. (2020, 09 25). Performance Audit on Mental Healthcare in Gujarat. Gujarat (General and Social), 5 of 2020. <https://resourcerepository.azimpremjiuniversity.edu.in/items/3427b0ec-76fd-4511-bcdf-c74f402ccff0/full>

continuity, relational care, social reintegration and respect for agency. For advocacy and reform, they provide concrete guidance on how India's mental health system can move beyond custodial logics toward care that is genuinely person-centred, community-anchored and rights-affirming.

5. Conclusion

This white paper examines the status of mental health infrastructure in Maharashtra, India, highlighting the challenges of a system that still relies heavily on large custodial institutions despite a global shift towards community-based care. It critiques the inadequate workforce, systemic problems within institutions, and the impact of social prejudice and stigma on the quality of care. The paper advocates for trauma-informed, gender-sensitive, and rights-based models of care, emphasising the need for community-anchored supports and the dismantling of custodial institutions. India's public mental health institutions starkly reflect entrenched systemic failures, from their overreliance on hospital-centric and (DMHP)-dominated approaches to chronic underfunding and exclusionary practices that sideline communities, users, and carers. Yet, as evidenced by decades of research, these challenges are not insurmountable; pathways for profound transformation emerge through community-based models and rights-aligned reforms.

Four decades after the NMHP launched in 1982, services remain "inadequate, inequitably distributed and poorly integrated," as Gangadhar et al. (2023) assert in their narrative review, attributing stagnation largely to "low, unstable and poorly targeted financing" for NMHP and DMHP. This funding shortfall manifests in administrative bottlenecks, such as inaccessible funds and uneven district-level rollout, leaving rural populations underserved despite policy ambitions like the Mental Healthcare Act, 2017.⁹² Systematic reviews of India's mental health response highlights accountability deficits, including limited community participation, minimal involvement of users and caregivers in oversight, and weak engagement by national human rights institutions, which hinder the translation of legal advances into practice. These institutional shortcomings perpetuate a custodial paradigm, where MHIs prioritise containment over recovery, echoing broader gaps in community integration documented in systematic reviews of task-sharing and village-level interventions.⁹³ Amid rising mental health

⁹² Gangadhar, B. N., Naveen Kumar, C., Sadh, K., Manjunatha, N., Math, S. B., Kalaivanan, R. C., ... & Chaturvedi, S. K. (2023). Mental health programme in India: Has the tide really turned? *Indian Journal of Medical Research*, 157(5), 387–394. https://doi.org/10.4103/ijmr.IJMR_2217_20

⁹³ Hans, G., & Sharan, P. (2021). Community-based mental health services in India: Current status and roadmap for the future. *Consortium Psychiatricum*, 2(3), 63–71. <https://doi.org/10.17816/CP92>; Verma, P., Sahoo, K. C., Mahapatra, P., Kaur, H., & Pati, S. (2023). A systematic review of community-based studies on mental health issues

burdens, exacerbated by economic stressors and crises, the urgency is palpable: without reform, public MHIs risk entrenching trauma rather than alleviating it.

At its core, India's public MHIs embody not inevitability but a failure of imagination, rooted in top-down, under-resourced structures that marginalise the very communities they serve. However, the literature reveals viable pathways: rapidly expanding community models, village case-finding, integration into maternal-child health platforms, and peer-support groups offer partial substitutes for hospital dominance. Task-sharing with lay workers under supervision, as in the MANAS trial, demonstrates feasibility for common disorders, while community-based rehabilitation fuses clinical care with livelihoods.⁹⁴ These alternatives counter NMHP/DMHP inequities by fostering inclusive governance, stable financing, and human-rights alignment, directly addressing the stagnation. Transformation demands urgency: with mental health needs surging, delay condemns millions to custodial harm. Yet possibility abounds. #CareNotCustody envisions institutions that heal, safe spaces prioritising consent, peer expertise, and family involvement; inclusive designs amplifying marginalised voices; and care ethics that reject punishment for solidarity. Feminist frameworks, valuing interdependence over isolation, provide the blueprint: redirect DMHP funds to grassroots networks, mandate user-carer boards, and embed rights monitoring. Programs like BasicNeeds India already model this, linking treatment to self-help groups. By heeding these calls, India can pivot from custody to care, building resilient systems that honour dignity today.

This research was undertaken after our own experiences of tackling systemic issues with mental healthcare quality and delivery issues in Maharashtra during COVID-19, a period that saw us confront the many challenges discussed above when providing community-based mental healthcare services. The purpose of this research is to highlight the need for care systems outside of institutional control that can replace asylum-style custody with dignified, peer-led support systems. Through toolkits, webinars, and advocacy briefs, #CareNotCustody amplifies models rooted in communities of care, demonstrating that community platforms can deliver psychosocial support at scale. By centring feminist ethics, emphasising relationality, mutual aid, and anti-carceral principles, our goal is to transform academic findings into tangible demands, such as budget reallocation from institutions to rehabilitation

among tribal populations in India. *Indian Journal of Medical Research*, 156(2), 291–298.

https://doi.org/10.4103/ijmr.ijmr_3206_21

⁹⁴ Sivakumar, T., Thirthalli, J., Kumar, C. N., & Basavarajappa, C. (2020). Community-based rehabilitation for persons with severe mental illness in a rural community of Karnataka: Methodology of a randomized controlled study. *Indian Journal of Psychological Medicine*. <https://doi.org/10.1177/0253717620971203>

linkages and build better systems that foreground mental healthcare as a fundamental right that must be affordable, accessible and of the highest quality.

ANNEXURE A: LIST OF ABBREVIATIONS

Abbreviation	Full Form
CHC	Community Health Centre
CRPD / UNCRPD	United Nations Convention on the Rights of Persons with Disabilities
DMHP	District Mental Health Programme
ECG	Electrocardiogram
ECT	Electroconvulsive Therapy
EEG	Electroencephalogram
GHPU	General Hospital Psychiatric Unit
ICCPR	International Covenant on Civil and Political Rights
ICESCR	International Covenant on Economic, Social and Cultural Rights
ICU	Intensive Care Unit
MHI	Mental Health Institution
MHIs	Mental Health Institutions
MHCA 2017	Mental Healthcare Act, 2017
MI Principles	UN Principles for the Protection of Persons with Mental Illness and the Improvement of Mental Health Care (1991)
NCW	National Commission for Women
NGO	Non-Governmental Organisation
NIMHANS	National Institute of Mental Health and Neuro Sciences

NMHP	National Mental Health Policy
NMHP 2014	National Mental Health Policy, 2014
NMHP (1982)	National Mental Health Programme (launched in 1982)
PHC	Primary Health Centre
RPwD Act	Rights of Persons with Disabilities Act, 2016
SHG	Self-Help Group
SMHA	State Mental Health Authority
Tele-MANAS	Tele Mental Health Assistance and Networking Across States
UDHR	Universal Declaration of Human Rights